

SUPER SHRED

FACT-CHECKED PRECLINICAL PROTOCOL & WORKOUT TIMING (MAX 2–3 DOSES / WK)

Target Form: 10mL Injectable Vial Reference | Frequency: Max 2–3 Doses Weekly | Pre-Workout Timing: 30–60 Mins Before Training (AM Only)

MANDATORY RESEARCH DISCLAIMER: This compound is NOT approved by the FDA or international health authorities for human consumption. It is formulated strictly for *in vitro* laboratory testing and preclinical animal research models. This guide synthesizes standard laboratory handling procedures, theoretical dosing structures, and chemical mechanisms for educational and scientific research purposes only.

1. PRE-WORKOUT TIMING GUIDANCE



OPTIMAL WINDOW

30 to 60 Minutes Before Exercise. Align administration so that peak systemic absorption coincides with the onset of training.



PHARMACOKINETIC PEAK

Albuterol $_{T_{max}}$ (peak concentration) occurs at **30–60 min**. Aligns peak bronchodilation, cardiac output, & lipolysis with session.



NOON CUTOFF (12 PM)

Hard Cutoff: 12:00 PM. Due to Albuterol's 4–6 hour half-life, administration past noon causes night-time tachycardia & severe insomnia.

CRITICAL SLEEP & INSOMNIA WARNING:

DO NOT ADMINISTER IN THE EVENING OR AT NIGHT. The inclusion of **ALBUTEROL (2 MG/ML)**—a potent Beta-2 adrenergic sympathomimetic stimulant with a 4 to 6 hour plasma half-life—causes sustained Central Nervous System (CNS) stimulation, elevated resting heart rate, and increased core body temperature. Late-day administration severely disrupts sleep architecture, causes acute sleep-onset insomnia, and hinders physical recovery.

2. FACT-CHECKED COMPOUND COMPOSITION & MECHANISMS

COMPONENT	CONCENTRATION	PHARMACOLOGICAL FACT CHECK & MECHANISM
L-Carnitine	400 mg / mL	Translocates long-chain fatty acids into mitochondria via CPT-1/CPT-2 enzymes for beta-oxidation. Non-stimulatory, but high doses support cellular energy output.
MIC Blend (Methionine / Inositol / Choline)	100 mg / mL	Lipotropic Support: Choline & Methionine facilitate VLDL synthesis and hepatic fat export; Inositol modulates cellular signal transduction. Non-stimulatory.
ATP (Adenosine Triphosphate)	50 mg / mL	Extracellular purinergic signaling agonist (P2 receptors). Exogenous systemic ATP has an extremely short intravascular half-life (seconds) and promotes vasodilation.
Albuterol (Salbutamol)	2 mg / mL	PRIMARY STIMULANT Selective Beta-2 adrenergic agonist. Accelerates thermogenesis, lipolysis via cAMP pathway, and heart rate. Half-life: 4–6 hrs. Main driver of insomnia/jitteriness.
Vitamin B12 (Cyanocobalamin)	1 mg / mL	Essential cofactor for methionine synthase and methylmalonyl-CoA mutase, supporting nucleic acid synthesis and RBC formation.

3. PRECLINICAL RESEARCH DOSING PROTOCOL (MAX 2–3 DOSES / WEEK)

RESEARCH PHASE	VOLUME / DOSE	WEEKLY SCHEDULE	ADMINISTRATION TIMING & GOAL
PHASE 1: INDUCTION	0.5 mL	2 Doses / Week (Mon & Thu AM)	30–60m Pre-Workout (AM): Assess baseline cardiovascular response, thermal shift, and CNS tolerance.
PHASE 2: STANDARD	1.0 mL	2–3 Doses / Week (Mon / Wed / Fri AM)	30–60m Pre-Workout (AM): Primary window for thermogenic evaluation and fatty acid utilization.
PHASE 3: HIGH YIELD	1.5 mL – 2.0 mL	2–3 Doses / Week Max (Mon / Wed / Fri AM)	30–60m Pre-Workout (AM): Maximum threshold evaluation. High risk of tremors, tachycardia, and insomnia if delayed.

4. 8-WEEK SCHEDULE & WASHOUT DESIGN

TIMELINE	WEEKLY FREQUENCY & TIMING	MONITORING & SAFETY FOCUS
Weeks 1 – 2	0.5 mL — 2x / week (30–60m Pre-Workout, Early AM)	Monitor heart rate, blood pressure, tremor severity, and sleep onset times.
Weeks 3 – 6	1.0 mL — 2 to 3x / week (30–60m Pre-Workout, Early AM)	Peak thermogenic window. Verify that no dose is administered after 12:00 PM.
Weeks 7 – 8	1.0 mL – 1.5 mL — 2 to 3x / week max (30–60m Pre-Workout, Early AM)	Final data window. Watch for signs of Beta-2 receptor desensitization.
Weeks 9+	MANDATORY WASHOUT (0.0 mL)	Cease administration for 4–6 weeks to enable full Beta-2 receptor resensitization.

5. FACT-CHECKED SAFETY & TOXICITY INDICATORS

- **Sleep Disturbance / Insomnia:** Albuterol's 4–6 hour half-life and central sympathomimetic stimulation directly inhibit sleep onset and REM cycles if administered late in the day.
- **Cardiovascular Effects:** Tachycardia, elevated systolic blood pressure, palpitations, and increased resting cardiac output.
- **Electrolyte Imbalance (Hypokalemia):** Beta-2 stimulation activates Na^+/K^+ ATPase pumps, shifting potassium into intracellular compartments. Potassium levels must be monitored.
- **Neurological Symptoms:** Fine skeletal muscle tremors (hands), acute anxiety, restlessness, and headaches.